

Long-Acting GCSFs

Original Effective Date: 6/2021

Revision Date(s): 6/2021; 9/2021; 5/2/2022; 06/13/2022; 7/28/2022; 10/26/2022; 12/5/2023; 4/17/2024; 7/1/2024; 2/17/2025; 1/1/2026; 3/16/2026

Review Date: 3/16/2026

Policy type: Medical Necessity

Line of Business: All G.E.H.A Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan, or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety, and tolerability.

Preference	Requires Prior Auth	Drug Name	HCPCS Code	Description
Non-Preferred	Yes	Neulasta	J2506	Injection, pegfilgrastim, excludes biosimilar, 0.5 mg
Non-Preferred	Yes	Rolvedon	J1449	Injection, eflapegrastim-xnst, 0.1 mg
Non-Preferred	Yes	Armlupeg	J3590	Unclassified biologics
Non-Preferred	Yes	Ryzneuta	J9361	Injection, efbemalenograstim alfa-vuxw, 0.5 mg
Non - Preferred	Yes	Fulphila	Q5108	Injection, pegfilgrastim-jmdb, biosimilar, (Fulphila), 0.5 mg
Non-Preferred	Yes	Udenyca	Q5111	Injection, pegfilgrastim-cbqv, biosimilar, (Udenyca), 0.5 mg
Non - Preferred	Yes	Ziextenzo	Q5120	Injection, pegfilgrastim-bmez, biosimilar, (Ziextenzo), 0.5 mg
Preferred	Yes	Nyvepria	Q5122	Injection, pegfilgrastim-apgf, biosimilar, (Nyvepria), 0.5 mg
Non-Preferred	Yes	Stimufend	Q5127	Injection, pegfilgrastim-fpgk (Stimufend), biosimilar, 0.5 mg
Preferred	Yes	Fylnetra	Q5130	Injection, pegfilgrastim-pbbk (Fylnetra), biosimilar, 0.5 mg

- Members must have documentation of a contraindication, failure, or intolerance to any of the preferred agents prior to approval of a non-preferred product.
- Preferred product requirements apply to all requests, both new starts and reauthorizations.
- Step Therapy applies to all overlapping compendia supported indications/regimens.

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

- National Comprehensive Cancer Network® (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; **OR**
- United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; **OR**
- Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:
 - Clinical Pharmacology (Strong For); **OR**
 - Wolters Kluwer Lexi-Drugs® (Level A); **OR**
- Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:
 - American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;
 - **Meeting abstracts and case reports are excluded from consideration;**
- Non-standard protocols may be approved based on unique clinical circumstances;
- Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network® (NCCN), or indication specific peer-reviewed literature;
- In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing Codes

Drug Name	HCPCS Code	Description
Rolvedon	J1449	Injection, eflapegrastim-xnst, 0.1 mg
Neulasta	J2506	Injection, pegfilgrastim, excludes biosimilar, 0.5 mg
Armlupeg	J3590	Unclassified biologics
Ryzneuta	J9361	Injection, efbemalenograstim alfa-vuxw, 0.5 mg
Fulphila	Q5108	Injection, pegfilgrastim-jmdb, biosimilar, (Fulphila), 0.5 mg

Udenyca	Q5111	Injection, pegfilgrastim-cbqv, biosimilar, (Udenyca), 0.5 mg
Ziextenzo	Q5120	Injection, pegfilgrastim-bmez, biosimilar, (Ziextenzo), 0.5 mg
Nivepria	Q5122	Injection, pegfilgrastim-apgf, biosimilar, (Nyvepria), 0.5 mg
Stimufend	Q5127	Injection, pegfilgrastim-fpgk, biosimilar, (Stimufend), 0.5 mg
Fynetra	Q5130	Injection, pegfilgrastim-pbbk, biosimilar, (Fynetra), 0.5 mg

References

1. Fulphila [package insert]. Biocon Biologics, Inc., Cambridge, MA. Available at: <https://dailymed.nlm.nih.gov/dailymed/fda/fdaDrugXsl.cfm?setid=9bfc4d3e-6120-7fec-f1d6-0b3743752034&type=display>
2. Fulphila. NCCN Cancer Guidelines and Drugs and Biologics Compendium. Available at: https://www.nccn.org/professionals/drug_compendium/content/
3. Fynetra. [package insert]. Kashiv BioSciences, LLC., Piscataway, NJ; Available at: https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/761084s000lbl.pdf
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19. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®). National Comprehensive Cancer Network, Inc. 2026. All rights reserved. Accessed March 10, 2026. To view the most recent and complete version of the guideline, go online to NCCN.org.

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Revision History

Date	Updates
5/2/2022	Added preferred products and recommended step-therapy options. Added Wilms tumor per NCCN, added H-ARS per PI/NCCN
06/13/2022	Added Fylmetra(pegfilgrastim-pbbk)
07/28/2022	Removed H-ARS as per scope
10/26/2022	Added Stimufend (pegfilgrastim-fpgk), Rolvedon (eflapregrastim-xnstm)
10/27/2022	Policy approved by P&T
12/5/23	Updated Step Therapy to Table; Added Universal Criteria; Addition of efbemalenograstim alfa; Reviewed References
4/17/2024	Policy adopted by G.E.H.A; document updated for Universal accessibility
7/1/2024	Ryzneuta HCPCS Updated
2/12/2025	Annual Review (update template, Universal Criteria)
2/17/2025	Removed med nec criteria, updated preferred product requirements; approved by P&T.
1/1/2026	Addition of Armlupeg (pegfilgrastim-unne) NTM drug (PI unavailable on 12/23/2025)
3/16/2026	Annual Review – approved by OH P&T.